

EMERGENCY PATIENT MEDICAL REPORT

■■ EMERGENCY CASE - IMMEDIATE ATTENTION REQUIRED

Patient Information

Name:	Robert Johnson
Age:	67 years
Gender:	Male
Patient ID:	EMRG-789012
Date/Time:	January 9, 2026 - 19:45
Arrival Mode:	Ambulance - Code Stroke

Chief Complaint

SUDDEN ONSET severe headache, confusion, and left-sided weakness.

Patient was found collapsed at home by family members at 18:30. Wife reports patient complained of "worst headache of my life" before losing consciousness for approximately 2 minutes. Upon regaining consciousness, patient was confused, disoriented, and unable to move left arm. Speech was slurred.

Time of symptom onset: Approximately 18:25 (80 minutes ago)

Last known well: 18:15

NIHSS Score: 14 (moderate to severe stroke)

Vital Signs (on arrival)

Parameter	Value	Status
Blood Pressure	190/110 mmHg	■■ CRITICAL HIGH
Heart Rate	88 bpm	Normal
Temperature	98.4°F	Normal
Respiratory Rate	18/min	Normal
Oxygen Saturation	96%	Normal
Glasgow Coma Scale	13/15	■■ Decreased

Neurological Examination

Mental Status: Confused, disoriented to time and place

Speech: Slurred, difficulty finding words (expressive aphasia)

Cranial Nerves: Left facial droop (CN VII), pupils equal and reactive

Motor: Left arm 1/5 strength, left leg 2/5 strength, right side 5/5

Sensory: Decreased sensation on left side

Reflexes: Hyperreflexia on left, positive Babinski on left

Coordination: Unable to assess due to left-sided weakness

STAT Laboratory Results

Test Name	Result	Normal Range	Unit
Hemoglobin	15.8	13.5-17.5	g/dL
WBC Count	13.2	4.0-11.0	10 ³ /μL
Platelets	185	150-400	10 ³ /μL
Glucose	156	70-100	mg/dL
Sodium	138	136-145	mEq/L
Potassium	4.2	3.5-5.0	mEq/L
Creatinine	1.1	0.6-1.2	mg/dL
PT/INR	1.1	0.9-1.1	ratio
PTT	28	25-35	seconds

■■ BRAIN CT SCAN (NON-CONTRAST)

CRITICAL FINDINGS - STAT NEUROSURGERY CONSULT



RADIOLOGIST INTERPRETATION:

FINDINGS:

- Large hyperdense area in the RIGHT BASAL GANGLIA region measuring approximately 3.5 x 3.0 cm
- Consistent with ACUTE INTRACEREBRAL HEMORRHAGE (ICH)
- Mild mass effect with compression of the right lateral ventricle
- No midline shift currently visible
- No evidence of herniation at this time
- No subarachnoid hemorrhage identified

IMPRESSION: ACUTE RIGHT BASAL GANGLIA HEMORRHAGE (HEMORRHAGIC STROKE)

Estimated volume: ~35 mL

Location: Deep right hemisphere (putamen/internal capsule region)

Risk: HIGH risk for expansion and neurological deterioration

Past Medical History

Chronic Conditions:

- Hypertension (poorly controlled, on medication irregularly)
- Type 2 Diabetes Mellitus
- Hyperlipidemia
- Chronic kidney disease Stage 2

Medications (Home):

- Lisinopril 20mg daily (admits poor compliance)
- Amlodipine 10mg daily
- Metformin 1000mg twice daily
- Atorvastatin 40mg nightly

Social History:

- Former smoker (quit 5 years ago, 40 pack-year history)
- Occasional alcohol use

- Sedentary lifestyle

Family History:

- Father died of stroke at age 69
- Mother with hypertension and diabetes

EMERGENCY DEPARTMENT DIAGNOSIS

ACUTE HEMORRHAGIC STROKE - RIGHT BASAL GANGLIA

Assessment: 67-year-old male with sudden onset severe headache, confusion, and left hemiparesis. Brain CT confirms acute intracerebral hemorrhage in right basal ganglia. Presentation consistent with hypertensive hemorrhagic stroke given history of poorly controlled hypertension.

Severity: Moderate-severe (NIHSS 14, hemorrhage volume ~35mL)

Etiology: Most likely hypertensive hemorrhage (chronic uncontrolled BP)

Complications Risk: HIGH - risk of hematoma expansion, increased ICP, herniation

■■ IMMEDIATE MANAGEMENT INITIATED:

- Neurosurgery STAT consult (on way)
- ICU admission arranged
- Blood pressure control initiated (target SBP 140-160)
- Repeat CT scheduled in 6 hours
- Intracranial pressure monitoring consideration
- Reversal agents if needed (none needed - normal INR)

PROGNOSIS: Guarded. Significant neurological deficits present. Recovery will depend on hemorrhage stability, response to treatment, and rehabilitation.